

REGIONAL CARDIOVASCULAR IMAGING
CENTER

DIAGNOSTIC REPORT

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NPI: 1234567890

Patient Name: Nick	Date of Birth: 1966-08	Age: 60 yrs
MRN: MRN-2026-00847	Exam Date: 2026-03-20	Report Date: 2026-03-21
Ordering Physician: Dr. Sarah Chen, MD, FACC	Accession #: CAC-2026-03201	Sex: Male

CT CORONARY ARTERY CALCIUM SCORING

CLINICAL INDICATION

ASCVD risk stratification; post-NSTEMI follow-up; statin therapy monitoring. Patient has known history of NSTEMI (2021) with PCI/stent placement to LAD. Currently on atorvastatin 40mg. Elevated Lp(a) noted on recent labs.

TECHNIQUE

Non-contrast ECG-gated CT of the chest was performed on a 64-slice multidetector CT scanner. Images were acquired during a single breath-hold. Coronary artery calcium was quantified using the Agatston scoring method. Contiguous 3mm axial slices through the heart were obtained.

KEY FINDINGS

- Calcified plaque present in the left anterior descending artery (LAD) — predominant vessel.
- Mild calcification identified in the left circumflex artery (LCX).
- No significant calcification in the right coronary artery (RCA).
- No pericardial effusion identified.
- No incidental pulmonary or mediastinal findings of significance.

	Agatston SCORINGS Score	Units	Interpretation
Total CAC Score	142	Agatston units	Elevated; 75th percentile for age/sex; moderate atherosclerotic burden
LAD (Left Anterior Descending)	118	Agatston units	Predominant vessel; consistent with prior PCI/stent

LCX (Left Circumflex)	24	Agatston units	Mild calcification; monitor
RCA (Right Coronary Artery)	0	Agatston units	No calcification detected

Risk Percentile Context: CAC score of 142 places this patient above the **75th percentile** for males aged 55-65 (MESA reference). Scores 100-400 indicate moderate atherosclerotic plaque burden with elevated 10-year ASCVD risk. This result is consistent with the patient's known high-risk cardiovascular history.

IMPRESSION

Moderate coronary artery calcification with total Agatston score of **142**, predominantly in the LAD (score 118), consistent with known ASCVD and prior PCI with drug-eluting stent. Mild calcification in the LCX (score 24). No calcification in the RCA. Overall score places the patient above the 75th percentile for age and sex, confirming high cardiovascular risk category. Findings support continuation and optimization of lipid-lowering therapy.

RECOMMENDATIONS

- 1. Continue high-intensity statin therapy (atorvastatin 40mg); maintain LDL-C < 70 mg/dL and ApoB <80 mg/dL.
- 2. Reassess CAC score in 3-5 years, or sooner if lipid targets are not maintained.
- 3. Discuss elevated Lp(a) in the context of these CAC findings with treating cardiologist; consider referral for advanced lipid management.
- 4. Continue antiplatelet therapy (aspirin 81mg) and ACE inhibitor as prescribed.
- 5. Maintain current high activity level — exercise capacity is a strong independent cardiovascular protective factor.

Interpreting Radiologist: James R. Hoffman, MD,
FSCCT
Board Certified — Diagnostic Radiology & Cardiovascular
CT
Electronically signed: 2026-03-21 | 14:32 PST

Reviewed by: Sarah Chen, MD, FACC
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